National Cancer Control Programme (NCCP) 00261 · clinician draft

Carboplatin · 21-day breast schedule

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Carboplatin AUC4–6 · 21-day schedule

Active medicines: Carboplatin

Emetogenic risk: High at AUC ≥4

Medicine	Dose / route	When
Aprepitant	125 mg orally on Day 1; 80 mg orally on Days 2 and 3	Once daily on each listed day Check interactions; follow the prescribed plan.
Ondansetron	16 mg orally on Day 1	Before treatment NCIS permits individual splitting to 8 mg twice daily or reduction to 8 mg once daily. Review QT risk, electrolytes and constipation.
Dexamethasone	12 mg orally on Day 1; 8 mg orally on Days 2, 3 and 4	Once daily on each listed day Confirm timing and diabetes/sleep advice. This is the dedicated carboplatin row.
Metoclopramide · duration requires prescription	10 mg orally three times daily on subsequent days in the NCIS row	The source row does not specify an end day or PRN qualifier Prescriber/pharmacist must specify duration and suitability, including renal function and neurological contraindications. Do not continue indefinitely.

Additional phase-specific support

- NCIS Table 1 allows clinician-selected ondansetron splitting to 8 mg twice daily or reduction to 8 mg once daily according to the individual patient.
- Review ondansetron QT risk and other QT-prolonging medicines/electrolytes.
- Check dexamethasone interactions, diabetes, sleep and infection effects; aprepitant/steroid interaction is already reflected in the specified row.
- Review metoclopramide renal function, neurological contraindications, duration and extrapyramidal risk; do not automatically stack with olanzapine.
- AUC reduction below 4 requires an individual prophylaxis review; history of emesis and patient factors may justify continuing stronger protection.
- No universal granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), antibiotic, antiviral or acid-suppression prescription is stated by the carboplatin protocol. G-CSF requires individual febrile-neutropenia risk review.
- Routine hypersensitivity premedication is usually unnecessary, but antiemetic prophylaxis is still required. Prior allergy/desensitisation is a separate specialist plan.
- Use the dedicated carboplatin AUC ≥4 row. It does not include routine olanzapine.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00261 v7 · p4 · NCCP NCIS antiemetic medicines v8 · dedicated carboplatin AUC ≥4 row, pages 4–5

Sources checked 2026-10-25 · SACTCheck v0.84.20 · Regimen-level drafts; prescriber and oncology-pharmacy verification required